

28 April 2023

Submission by the Defence of No Case To Answer

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Housekeeping

Friday, 28 April 2023

(10.30 am)

(Delay in proceedings)

(11.00 am)

(In the absence of the jury)

APPLICATION Submissions by MR MYERS

MR JUSTICE GOSS: Yes, Mr Myers.

MR MYERS: My Lord, we're grateful for the time this morning just to prepare to start the submission. The court knows that we make a submission of no case to answer and I'll set out the parameters of that shortly.

Your Lordship will have the defence submission and bundle --

MR JUSTICE GOSS: Yes.

MR MYERS: -- the prosecution response, for which we are grateful. We appreciate that has been dealt with in quite a short compass of time, we understand that, inevitably.

There were two authorities that I sent through to your Lordship, but I'm grateful to Mr Maher, who's actually dealing with the printing of them --

MR JUSTICE GOSS: Well, I gather that my clerk said could he print them off but I don't think they need to be because you're only referring to discrete paragraphs --

MR MYERS: It is --

MR JUSTICE GOSS: So don't print the whole thing off.

MR MYERS: And it's only to support a proposition that I put to the court that in its basic terms, I anticipate, is not controversial. It's a matter of applicability to the current situation. They are authorities, the general circumstances of which the court will be aware of.

MR JUSTICE GOSS: Can someone stop him printing off?

MR MYERS: We'll see if we can head him off at the printer my Lord. Thank you.

Subject to observations from your Lordship, or any direction as I proceed, I'm going to follow the submission as we have set it out, but seek to do so with brevity where I can, there are some points that I need to elaborate and enlarge upon, and also at appropriate points to deal with the principal matters raised by the prosecution.

MR JUSTICE GOSS: Yes. That would be helpful. As you would expect, I've read both sets of submissions. I obviously haven't delved into the authorities in detail. You can refer me to anything as you see fit and as you wish, but take it that I have the basic respective submissions.

MR MYERS: I'm grateful, my Lord.

We make the submission primarily in the case of four counts where air embolus is alleged to be the sole cause of death or collapse. Those are counts 1 -- I set these out now so all can follow the purpose of the submission: Count 1, the case of [Baby A], which is an allegation of murder.

Count 2, which is [Baby B], an allegation of attempted murder.

Count 4, [Baby D], allegation of murder.

And count 16, [Baby M], an allegation of attempted murder.

Air embolus is alleged to be the sole cause of death in those counts.

We have then set out in paragraph 2 those counts in which air embolus is a cause of death or collapse. They are as follows: In count 3 [Baby C], which is an allegation of murder, and in that count Dr Dewi Evans has alleged air embolus and also air via the NGT, the nasogastric tube.

Count 5, the case of [Baby E], an allegation of murder. Dewi Evans has also alleged trauma in addition to air embolus.

Count 12, the case of [Baby I], which is an allegation of murder. There are a number of events there and the events on 13 October, the 14th and the night of the 22nd and 23rd all, to one extent or another, involve an allegation of air embolus, although not the sole allegation.

Count 17, which is the first count dealing with [Baby N], attempted murder. Air embolus has been alleged by Dewi Evans.

Finally, in count 20, [Baby O], murder. Air embolus is the mechanism alleged, although there are other mechanisms alleged to contribute or be responsible, and those have been set out in the argument.

MR JUSTICE GOSS: Yes.

MR MYERS: Our submission is that medical evidence is fundamental to these allegations. I'm going to return to that shortly, but we submit that the scientific basis for air embolus is so weak that it fails to provide the level of reliability required to support these allegations. And for that reason, we submit that a jury, properly directed, could not properly convict on it.

We have set out the relevant part of the R v Galbraith and the element that applies is what's sometimes called the second limb of Galbraith, we maintain, which is that the evidence is inherently so weak or vague, or in this case we say inherently so unreliable on a scientific basis, that it cannot properly support the allegations that are made, which are central to these allegations. That is why we say on counts 1, 2, 4 and 16 there is no case to answer. And in counts 3, 5, 12, 17 and 20, we respectfully submit that, insofar as this evidence has been relied upon to support a conviction, the jury should be directed to disregard it.

Before I go further with the body of our submission, I'll deal now with a number of points raised by the prosecution to assist the court and I make apology if in doing so I haven't digested the full import of any aspect of them. I will wait to hear if it's different from how I see it.

MR JUSTICE GOSS: Exactly, yes. This is in anticipation, so it's your understanding, so obviously you'll have a right of response.

MR MYERS: I'm grateful. There are three matters that I am going to deal with at this stage, which run through the prosecution response. The first is the propriety of the submission of no case to answer at this point and what's said about that; the prosecution say it is an abuse.

The second is whether the medical evidence alone is enough to base this submission upon given the circumstantial evidence that the prosecution identify the fact there are other counts, there are other matters to take into account, and therefore even if there is merit or any weight to the defence submissions, they do not dispose of those counts. Problems with the medical evidence, in other words, is not fatal to the count insofar as there are any problems recognised, and they would dispute that there are problems. We recognise that.

The final point that I'll deal with at this stage, and I come to it, is the position of Dr Hall, who has featured in the prosecution response as a reason to support their position in resisting the defence application. I'll go through them in that order because that's really the way they come through in the prosecution argument.

First of all, the question of the defence making a submission of no case to answer and the criticisms by the prosecution, in particular, are at paragraphs 7 and 8 of their argument. They culminate in the proposition that in effect this is an abuse of the process by the defence; that appears at paragraph 25.1. That's their first ground of resistance to this application.

We don't accept any of that for a moment. The purpose of this application, absolutely plainly, from its terms and its content is the sufficiency of medical evidence, in particular whether or not the basis for what might be called a theory of air embolus is sufficiently well-established and reliable to support the allegations where they are made in the different counts. That's the purpose of this and our submission is it falls, when looked at collectively, a long way from the level of strength and reliability that scientific evidence should have in a case like this.

That submission is based entirely on the evidence as it has unfolded during this trial.

I could not possibly have stood here at the start of this trial and turned to 500 pages of evidence, or parts of those pages, in support of the submissions that we make on behalf of the defence if this trial had not taken place and we had not got to this stage. The basis for this application is the state of the prosecution evidence at the end of their case.

We submit that it is extraordinary -- this isn't intended to be rude -- but badly judged to criticise the defence for an entirely proper application that they are entitled to make as a part of the trial process at the conclusion of the prosecution case, and we respectfully submit, on the basis of the way the prosecution evidence on air embolus has unfolded, it cannot

possibly be a matter of surprise that there is heavy criticism of that. That may not have been possible at the outset but most definitely is, we submit, well-grounded now. And we are compelled to observe that it simply isn't right to talk about abuse when the defence make use of the proper entitlement to critique the evidence on the basis of how it has unfolded.

If it is the view that even at the outset this evidence would not have been admissible, that could only add weight to the application that we make. But this isn't about admissibility at this point. We have made it very plain: this is about weight, the court having now heard this evidence. And our submission is the evidence of air embolus is inherently so weak and inconsistent that it cannot support a conviction.

That takes me to the second broad point as we perceive it, raised by the prosecution, which is that criticisms or shortcomings, if there be any, in the medical evidence alone are insufficient to meet what's required in this submission and the test the defence must satisfy.

In particular, your Lordship will find reference to that, and the way the prosecution deal with it, in paragraphs 21 to 23 of their response, which look at the interplay, although very briefly, but we understand why of course, between different counts on this indictment -- that's a matter that will have to be considered at some point if not now, but at some point -- other evidence, circumstantial evidence, and other factors by which they say Lucy Letby is implicated in these deaths.

So I deal now with that, that whatever the state of the medical evidence, that isn't enough for the defence to bring this argument. We submit that isn't right, respectfully submit that isn't right in the particular circumstances of these allegations.

In making that submission, my Lord, we start with the authority of Holdsworth, which the prosecution dwell upon in their response and go through the facts to show how they are different from this case, and of course they are different from this case. Serious as they are, they are certainly not on the scale we're dealing with here. I don't mean on the individual case but in terms of the number of counts looked at in the evidence.

Holdsworth [2008] EWCA Crim 971, at paragraph 12 of our argument.

We cite it because in that paragraph the Court of Appeal made very clear the high requirement, we submit, of medical evidence. What was said was this:

"Conclusions of medical experts on the cause of an injury or death necessarily involve a process of deduction, that is inferring conclusion from given facts based on other knowledge and experience. But particular caution is needed where the scientific knowledge of the process or processes involved is or may be incomplete. As knowledge increases today's orthodoxy may become tomorrow's outdated learning. Special caution is also needed where expert evidence is not just relied upon as additional material to support a prosecution but is fundamental to it."

That's the starting point of what we say about the law and the high requirement for reliability with scientific evidence. That isn't where we end and I'll return to where we go with that in due course. But this relates to what the prosecution say in two ways.

First of all, that where there is medical evidence, as the court said in Holdsworth, there is special caution to be exercised in the way that it is approached. We don't go on from that to suggest there's any particular complexity in that concept. The courts are well-used to those areas where particular caution is urged in other parts of the law. But it's important that it says that.

Secondly:

"That is particularly so where the evidence is fundamental to the allegation."

The prosecution in this response have gone into other details and circumstantial evidence and we understand that. But the medical evidence, we submit, is front and centre with these allegations and that is why we spent most of the last 6 months dealing with it in, at times, minute detail in this trial, with a succession of experts that would be hard to parallel, I suspect, in any other case.

If a mechanism is alleged in this case and the evidence is not sufficiently strong to support that, we submit that is a fundamental problem. A reason for that is that in this case, causation and the issues relating to that are firmly rooted in medical explanations. For example, this isn't -- I don't put this as a lurid explanation, but in a case, for example, of a gangland murder, an example may be given of a missing body case. The idea that there might be some medical cause or any medical basis for what's taken place as an explanation for the missing body or what happened is implausible unless, for example, there's a good basis for that. In those cases, a case like that and many other cases, of course circumstantial evidence may

weigh the more heavily and it may be the only evidence available.

But in this case we're dealing with a hospital setting, babies, many of whom are unwell or very fragile physically, as a starting point and certainly at risk of deterioration so far as their condition is concerned.

Therefore it isn't a case where it can easily be assumed that there must be deliberate harm in the absence of the prosecution being able to establish what harm there was.

That assumption cannot apply.

In medical cases, we submit, and this is why we have put the two authorities that we do before the court, and I'll go to it shortly, in cases that might be described as having a pre-eminently medical causation, a cause may not be identified, but in no way does it follow that culpable harm should therefore be inferred.

Of course that's a question of degree, we recognise that. Sometimes the extent of circumstantial evidence may be so great that it still calls for an explanation.

It may well be that is where the prosecution end up, if I can put it that way, with this application. We submit that is a significant shift in the case we've been dealing with up to this point where the mechanisms have been crucial because in the absence of them it's simply unascertained and may in fact be a medical cause unidentified, however many times that happens. It could be the same cause unidentified. It has been crucial to this prosecution to advance explanations because of the circumstances and they have sought to do so and most definitely presented the case in that way.

We listened closely to the opening, as your Lordship did, and again the predominance lay on the medical evidence and what it went to prove and that is what a large part of this trial has dealt with.

The authorities we sent through, once we had received the prosecution application, Cannings and Henderson, are authorities we anticipate the court will be familiar with, although there's obviously some detail in them. Both of them involve infant death in circumstances of alleged homicide.

In the case of Cannings a multiple death, although again, I observe, not to the scale we deal with in this case. And, in the case of Henderson, what's called -- I apologise for any

inelegance -- a baby-shaking case; that's how it's described by the Court of Appeal.

The reason that we cite those two authorities is they support what we say about the dangers of working on the basis that if the prosecution haven't identified a mechanism one can therefore infer deliberate harm.

In *Cannings* we take your Lordship to paragraph 13.

MR JUSTICE GOSS: Just give me a moment because I was only alerted to this, as you'll appreciate, minutes before the hearing started. I'm not being critical. I've been dealing with other things as well.

(Pause)

So we're looking first of all at *Cannings*?

MR MYERS: Yes.

MR JUSTICE GOSS: I'm getting that up now. I have it.

MR MYERS: It's the proposition or, rather, the statement of the court at paragraph 13 we go to. The court was dealing with what inferences can be drawn in what's called a sudden infant death case in terms of the explanations that are provided and the alternatives in the absence of full explanation.

We start at the second sentence, the first sentence follows on from that discussion, but it's where the court says this:

"Whether there are one, two or even three deaths the exclusion of currently known natural causes of infant death does not establish that the death or deaths resulted from the deliberate infliction of harm. That represents not only the legal principle which must be applied in any event, but in addition we shall see at the very least it appears to us to coincide with the views of a reputable body of expert medical opinion."

We don't go into the expert medical opinion in that case, it's the legal principle that we rely upon.

We submit that in the case of *Henderson*, some years later, which is at 2010, England and Wales Court of Appeal, Crim 1269, that principle or its sense is found and restated by the court at paragraph 44. We'll also go to 78. But at paragraph 44 the court said:

"We must recognise the limits of medical science and in particular that there may be events, deaths or symptoms which

are unexplained and unforeseen. Further, any conclusion must acknowledge the importance of the burden of proof in the context of cases such as these."

Cases such as these are cases turning on medical expert evidence:

"It is not for the defence to provide any explanation. The mere fact that it is unable to do so is not of itself a sound basis for concluding that the prosecution's evidence is correct."

We submit that is not the same words as in paragraph 13 of *Cannings*, but it approaches the matter -- the same matter in a different way. And I look next, my Lord, and take your Lordship to paragraph 78, which is a statement really specific to the case but it touches again on the principle:

"Further it is important to note that [this is the trial judge] stressed the defence was under no obligation to advance any particular theory as to cause of death and the rejection of the theory of spontaneous seizure did not of itself mean that the jury was required to accept the prosecution case and convict."

Of course, there we recognise that's the court identifying how a jury can be directed to deal with cases where medical evidence is textured in a way that there are decisions to be taken as to its strength or where alternative causes have to be considered.

But we submit that looking at the authorities, and I've only gone to them briefly, but simply to support the proposition I advance to your Lordship, that in a case with medical evidence, there may be an unidentified cause and that if the case proceeds on the basis that although the prosecution haven't identified a cause, nevertheless in some way that remains an issue, and that doesn't dispose of the prosecution case. The danger is that the burden then falls on the defence to meet that in some way.

In fact, to assist, it seems to me there are two propositions there. The first -- this seems to be where we get to with the argument I advance in light of this response, the first is that the prosecution evidence on cause is fundamental in this case and the way this case has been presented.

If it is not of sufficient strength to be reliable in the way scientific evidence should be then the counts that rely upon it, or those parts of the counts that rely upon it, are fatally undermined when the test of *Galbraith* is applied.

The second proposition in meeting this response from the prosecution, relying upon the principles identified, we submit, from the Court of Appeal, are that there is a danger in medical

cases of, in effect, reversing the burden where the prosecution have sought to advance, as they have here, medical evidence, which in fact cannot go to establish cause properly in the way the prosecution seek to establish but, we submit, if the case then proceeds, with the defendant in effect called upon to explain what has happened when there may be an unidentified medical cause.

That's why we submit it's particularly important for the court to keep in mind the setting of these allegations as against the generality of cases where it may be more easily said by the prosecution, well, there's a lot of circumstantial evidence here, what on earth is this unidentified cause? A perfectly healthy person has gone missing, we don't know what's happened, there's no medical issues to look at in particular, in the absence of any explanation, there's a very good basis for considering some sort of guilt or harm.

That doesn't follow in this case, we submit, and so for that reason seeking to meet the defence submission by pointing to other bits of evidence elsewhere neither reflects the way in which this prosecution case is advanced nor in fact, we respectfully observe, the way the evidence should be analysed at this point. So that's what we say with regard to the prosecution submission that there is evidence outwith the medical evidence that can compensate for any deficiencies.

MR JUSTICE GOSS: Just so I understand this, you are therefore saying that because of the nature of the evidence in relation to air embolus, no consideration should be attached to any other evidence because the cause of death cannot confidently be relied upon, the asserted cause of death cannot be relied upon, therefore the court should not look at any other evidence just to indicate, to assist in relation to a potential natural or unnatural causes of death? Is that what you're saying?

MR MYERS: The order in which that's dealt with is significant. We submit that if the medical evidence does not establish cause as alleged (overspeaking) --

MR JUSTICE GOSS: If the medical evidence has to establish cause of death and if it doesn't then one doesn't look at any other evidence, we just disregard any other evidence?

MR MYERS: On that count, yes. Most particularly, when dealing with the air embolus counts, that ends them entirely. There are other causes alleged or other possibilities on the other counts, the other five counts --

MR JUSTICE GOSS: Oh, I understand that.

MR MYERS: But on those particular four counts, yes, our submission is that this case has been powerfully based upon the allegation that this is an air embolus. The prosecution, in effect, are saying: we have the answer, we can say what this is, and upon that basis we submit that's what it is. Because in the absence of that, we submit, the evidence is not strong enough to support what's alleged. There may be various circumstantial matters but they're not decisive and they're not strong enough. It's the air embolus evidence that is pivotal and has been throughout.

MR JUSTICE GOSS: I understand that is the submission.

MR MYERS: That's the way we've put that.

MR JUSTICE GOSS: Can I then at this point raise a question I will ask: is it in dispute that air embolus can cause death?

MR MYERS: We haven't suggested it cannot cause death. As a fact, air embolus is capable of causing death. But we observe, without any sense of glibness whatsoever, that so are very many things, and seeking to tie the case to air embolus is a very specific mechanism. And the fact of death is in no way probative, we submit, of air embolus, and indeed that's one of the issues and one of our criticisms, that there's been great inconsistency on that point.

MR JUSTICE GOSS: I understand that.

MR MYERS: But as a possible mechanism, yes, of course it's recognised, if it's proved sufficiently, then of course it can be a mechanism of death, yes. We haven't taken issue with that and we don't.

MR JUSTICE GOSS: Okay. Sorry, I interrupted you. Please carry on. I just wanted to be clear in particular about what you were saying about the air embolus counts and where it's either the sole -- said to be the sole cause of death or a cause of death.

MR MYERS: Yes. Well, it's well-established and recognised, air embolism in principle can cause death. The question is whether the -- this argument at this stage of the case is whether the expert evidence on which that mechanism has been supposedly identified and applied -- the submission is it's so poor, it's inherently unreliable, that it doesn't support that finding.

It would be like saying, of course, that a bullet can cause death but the expert evidence on ballistics is so poor you can never actually prove that's what's happened.

Crucially also, I should say this, my Lord, part of that, twinned with that is the fact that it isn't accepted that air embolus is identified in these cases.

As a mechanism it can cause death, but there's certainly no concession that that is what's happened or what accounts for any piece of evidence in this case. That's tied up with the application that we make because we submit that where the expert evidence has sought to identify characteristics from which air embolus can be diagnosed, if it can be, they are fundamentally flawed as well.

So as a matter of principle, of course it can cause death, but we submit that's as far as that gets on this evidence.

MR JUSTICE GOSS: Right, thank you.

MR MYERS: Can I deal next with the third matter, third principal matter we submit the prosecution raise, which is the position of Dr Hall. We submit that for the purpose of this submission, that is entirely irrelevant.

It is not evidence in the case, it is not before the jury, it hasn't featured in the trial. It may never feature in the trial. That doesn't indicate what's going to happen with Dr Hall, but it may or may not.

The court will appreciate that it has to consider the evidence as it's presented to the court now at the close of the prosecution case.

His evidence plays no part in this application. The criticisms that Dr Hall has made of air embolus in this case, the criticisms that he's made of prosecution experts alleging air embolus are not matters we have sought to rely upon and we can no more seek to rely upon them to support our argument than, curiously, the prosecution regard Dr Hall as an expert on air embolus and seek to rely on it on their behalf. It is, respectfully, misconceived to try to use a defence expert who hasn't been called to meet a criticism of the prosecution evidence at this part of the case and it is a surprising recourse to take, and surprising because it's not in evidence and a basic feature of a submission of no case to answer is it looks at the evidence before the jury.

It's surprising because, so far as we understand, and the court may have perceived, the prosecution in general do not accept either large parts of what Dr Hall says or his conclusions. And it's particularly surprising because the prosecution vigorously resisted the introduction of Dr Hall's evidence during their case, absolutely resisted it, and sought a ruling from your

Lordship to that effect. It ill behoves them now, having in fact sought to prevent it from featuring in the case at this point, and it not featuring, to then seek to gain what they perceive to be some benefit by a reference to it when dealing with a defence application.

So we submit it plays no part in this, it could play no part in it, and it should play in part in it.

We also add this, just in parentheses, keeping in mind the criticisms made about the objective of the defence application. We applied for Dr Hall to give evidence at the start of this case. Your Lordship will have seen the criticism that somehow this was all mapped out by the defence when we began this application. In fact, even if it was, that wouldn't matter. But as it happens, it was the defence who sought Dr Hall to give evidence so the matter could be contested in detail as we proceeded, believing there would be a contest on the evidence, and there most certainly has been, but without Dr Hall participating in it.

But that observation takes me back to the first part of the response to the prosecution response, which is that this is an entirely proper application to make on the basis of the evidence at this part of the case, but Dr Hall can play no part in that we submit respectfully.

My Lord, those are the principal responses to the prosecution response. Of course, underlying their response in general is the argument, and I don't seek to be able to do justice to what Mr Johnson would say, but we perceive the argument that in any event any weakness is just a matter of the type of weakness a jury can deal with in the course of a case and be directed to do so appropriately and therefore what the defence complain of doesn't amount to what their submission relies upon.

So perhaps keeping that basic response to the defence in mind, and that basic exercise that I respectfully submit your Lordship will conduct in weighing up what is the way to deal with any perceived deficiencies in the evidence, perhaps I can then return to the main body of the submission.

MR JUSTICE GOSS: Certainly.

MR MYERS: Not all of it, that's unnecessary, but just to illustrate some of the points that we make and why it is that we say this evidence is just fundamentally unreliable. The authority of Dlugosz, 2013, England and Wales Court of Appeal, Criminal 2, our paragraph 13, states what is now in fact incorporated in the Criminal Practice Directions, which is that

it is essential to recall the principle, which is applicable, namely in determining the issue -- this was of admissibility:

"The court must be satisfied that there is sufficiently reliable scientific basis for the evidence to be admitted."

The Criminal Practice Direction at paragraphs A.4, A.5 and 19A.6, which is in the defence bundle, pages 1 to 4, sets out the approach to that, and of course Dlugosz, which was a case turning on low copy DNA, was looking at the admissibility of evidence.

As we submitted, we are not arguing admissibility here, but we submit that the type of factors identified in the Criminal Practice Direction in particular at paragraph 19A.5, are the type of factors to bear in mind when the court comes to consider the strength or reliability of scientific evidence in this case. They are set out at (a) through to (h). Although I may make some reference to specific ones as we go ahead it's probably unnecessary for me to read out everything it says there, but it is questions like the extent of the data on which an expert relies, that's at 19A.5(a), how inferences are made, how results are relied upon, input by others and peer review, that's at 19A.5(d), questions of evidence falling outside the expert's sphere of expertise, which we submit looms large in fact when looking at the prosecution experts, and completeness of information available to the expert, the consideration of a range of expert opinion, that's at (f) and (g).

We note also, significantly, at paragraph 19A.6, potential flaws, as they're described in the Criminal Practice Direction, flaws that may mean that the opinion is less reliable. For example, 19A.6(a):

"Being based on a hypothesis which has not been subjected to sufficient scrutiny including, where appropriate, experimental or other testing or which has failed to stand up to scrutiny."

We pick that one out in particular because on a number of occasions when questioned, prosecution experts, certainly in the cases of Dr Evans, consultant paediatric, Dr Bohin, consultant neonatologist, and Professor Arthurs, consultant paediatric radiologist, have all at certain points said, "We just don't have the studies or the data to answer that question". For example, how long it would take for how much air to have a particular effect. And we have identified examples where experiments have been made on adults, data from adults, experiments from dogs or rabbits, that type of response. I will go to one as an illustration in due course.

We submit that while there may be some academic sympathy with an expert trying to grapple with a question for which there is insufficient data because those experiments cannot be conducted, that is a problem in the halls of scholarship. But when we are in the environment we are in now, the implications of that change and we are, of course, dealing with questions of what can be proved. And the fact there may be inabilities to conduct experiments which would give the answer should not be an answer to an insufficiency of data or empirical evidence. And the flaw described in 19A.6(a), we submit, chimes precisely with that.

I will turn to the experts very briefly -- very shortly, sorry -- and deal with them, knowing your Lordship will have looked at the detail here and has, of course, listened to this evidence during the course of the trial. We know that.

There's one passage in a way which, we say, absolutely encapsulates -- there's a number of passages but there's one I'm going to go to now which encapsulates the problem with air embolus in this case.

I will take your Lordship there. It comes from the evidence, in fact, of Dr Sandie Bohin, the prosecution neonatologist. It's at page 410 of the bundle. I'll give your Lordship time to get there of course.

It's evidence that she gave in the case of [Baby N], count 17 on the indictment. It's at page 410.

MR JUSTICE GOSS: I have 410. Thank you.

MR MYERS: It's also -- this is referred to in the Sandie Bohin section of our submission at paragraph 47 as an example of where finally we get to with Dr Bohin and what we say is the unacceptable levels in the variation of skin discolouration that she maintained could assist in identifying air embolus.

At this point, and it's at internal page numbering 155 through to 156, and she was asked the question at line 16 at 155:

"Question: Across the whole of this trial, we don't have a constant by which we can measure air embolus, do we?"

I should say this was in the context of description of discolouration. Your Lordship will see that. In fact, it will help just to look at line 13, just above that, I asked Dr Bohin:

"Question: So far, we don't have any constant description of any discolouration, do we?"

"Answer: Not in this case."

Pausing there, we've identified in these submissions how actually the starting point in this case was that there was a specific discolouration advanced, most clearly by all the experts who gave evidence upon this, which is that description in the Lee and Tanswell paper, and I'll return to that. But that was the starting point. In fact various discolourations were then used as evidence of air embolus, which led to this, towards the end of this issue being explored in evidence:

"Question: We don't have a constant description of any discolouration, do we?

"Answer: Not in this case.

"Question: Across the whole of this trial we don't have a constant by which we can measure air embolus do, we?"

"Answer: I think what's come out across the whole of this trial is a huge variation on the types of skin changes seen by the parents, practitioners, nursing practitioners and doctors of these children. And certainly the medical and nursing personnel are very clear they've not seen changes like this before or since but that the changes appear quite graphic. So although there's not one single thing you can say is definitely pathognomonic of air embolus, that's a bit like saying all chickenpox rashes are different [as read]."

And we then went to chickenpox. But it's --

MR JUSTICE GOSS: "All chickenpox rashes are not" --

MR MYERS: "... are not different."

But it's what she said at the beginning of that answer that we say captures the problem:

"Answer: I think what's come out across the whole of this trial is a huge variation on the types of skin changes seen by parents, practitioners..."

And so on. That sounds very much like the trial process being used as a learning process and a source of data, what's come out in the course of the trial is...

That is a clear example of constructing a theory in the light of evidence that's come out. That is ex post facto theorising, utterly self-serving. I don't say that to be rude to Dr Bohin but that's what's happening.

And it's fundamentally wrong because, first of all, that is based upon a presumption before the analysis had been applied that we are dealing with air embolus. What she's actually saying there is: well, these are all cases of air embolus, I know I began by saying in Lee and Tanswell that's the description, or agreeing that's the description we have, and that was most relevant at the start of the case with [Baby A] and [Baby B], but in effect what's happening is she is saying, well, now that we have seen all these cases of air embolus we now know it presents with very varied discolourations.

So first of all, it indicates a presumption that we're dealing with air embolus and then seeking to justify that and, secondly, it shows just how unreliable this aspect, for example, of the expert evidence is because in effect she is ex post facto constructing the air embolus theory at the end of the case, having, we observe, been asked at the beginning as to the factors that define air embolus, asked in cross-examination, it wasn't set out before then, but asked in cross-examination, and discolouration in keeping with that identified in Lee and Tanswell was the starting point.

By the end of it, this expert is saying:

"Answer: I think that what's come out across the whole of this trial is a huge variation in the types of skin discolourations."

Well, that sounds, as we say, that the trial has almost been used as a source of self-serving evidence to prove what air embolus is because it has come out: actually, look at all these air emboli and what different colours they all have. We submit that's one illustration but one that gets to the nub of the shortcomings of this evidence and why it is so unreliable.

The position of the experts is dealt with by Dewi Evans at paragraph 19, as I've indicated. Knowing your Lordship will have been through this material and listened to the case, it's unnecessary for me to set out everything that's here. In short, like the other experts in this case, he has no clinical experience of diagnosing air embolus. He simply doesn't.

MR JUSTICE GOSS: One thing is that air embolus is rare.

All the experts have said air embolus is rare.

MR MYERS: Yes.

MR JUSTICE GOSS: So it may be very difficult to find anyone who has direct experience and knowledge of air embolus, which is unquestionably air embolus. Now, does that mean that therefore one cannot rely on any of the circumstances in relation to the

collapses of the babies which an expert says is consistent with an air embolus?

It cannot be relied on.

MR MYERS: Can I just clarify, does your Lordship mean by circumstances the medical circumstances around it or the --

MR JUSTICE GOSS: Just generally. This is one of the difficulties. All we essentially have in relation to observable changes to someone who is suffering an air embolus in life is this description from the Lee and Tanswell paper. This paper cites no more than five or six cases out of 50 where discolouration was present.

So the starting point there is that it is about 10% of the cases where there was any discolouration. Right?

MR MYERS: Yes.

MR JUSTICE GOSS: Of these, only one instance described the presentation that was regarded as the defining discolouration of air embolus at the outset of this case, this is you, and that description was:

"Blanching and migrating areas of cutaneous pallor were noted in several cases and in one of our own cases we noted bright pink vessels against a generally cyanosed cutaneous background."

So that is the extent of the evidence in relation to discolouration.

MR MYERS: Yes, it is, my Lord, and that is why we submit it is more crucial, not less crucial, for it to be construed restrictively in how it's applied because where there's uncertainty in medical or scientific knowledge, or where it's at the boundaries of a particular area, and perhaps uncertainty is the better way of putting it here, it's important to be, we respectfully submit, more careful with the interpretation and application of what learning there is than to say, well, this is very difficult, so there's going to be a degree of latitude that might not otherwise be acceptable. I'm not suggesting that follows from what your Lordship says, but there's a danger that lack of specificity justifies vagueness or imprecision from that point on.

We submit that where a mechanism or a scientific theory is applied in a criminal trial, it has to be sufficiently reliable. If there is limited data available and limited experience then, insofar as there are characteristics that can be identified, at the very least they should be applied rigorously.

Our submission, a large part of our submission is based not just on the fact that the starting point is not strong, we say, not strong because none of these clinicians actually have personal experience, and we say not strong because there is actually very little research and available data. But an important part of the criticisms we make is that, that having been acknowledged, from that point onwards in notable circumstances there have been constant variations to meet the exigencies of the evidence.

And where we get to with Dr Bohin and [Baby N], where ex post facto she incorporates a degree of variation into how air embolus can manifest itself based on her experience in looking at a trial in cases that she is treating as air embolus before it's been established on the basis of discolouration, that goes to show the dangers of that approach.

So with the authorities, Henderson, which talks about:

"Today's orthodoxy gets replaced by tomorrow's new learning."

And other cases looking at what is at the boundaries of knowledge, it is crucial to at least be -- for experts to be principled in their application of it.

I don't mean anything disreputable: by principled I mean stick with the principles we have.

Our submission is that the principles in the first place are weak and the experience is weak. But even if that were to be regarded as an unavoidable inevitability in the trial, and we say respectfully it shouldn't be, even if it were, the way that they have then been applied afterwards, in particular by Dr Evans and Dr Bohin, goes way beyond the acceptable limits of what can be done with that starting material and has become strikingly inconsistent or variable or unreliable.

MR JUSTICE GOSS: And so unreliable, you say, and inconsistent, that there is not a basis in respect of the counts in question -- let's just look at the ones where it is said to be the sole cause -- not a basis upon which a jury could be invited to conclude that there is sufficient evidence in the case as a whole to found a conviction?

MR MYERS: Yes, we do submit that because the scientific theory is unreliable --

MR JUSTICE GOSS: I understand that.

MR MYERS: -- and that's the test.

MR JUSTICE GOSS: Yes, that's it.

MR MYERS: With regard to evidence that can exist for air embolus, may I just take your Lordship, following through from the enquiry your Lordship raised, to page 509 of the defence bundle, which is the section of the Lee and Tanswell article that we've seen on a number of occasions, but it isn't the descriptions of colour that I take your Lordship to.

The bottom right corner, 508 on the top left, but the bundle numbering is 509. As your Lordship has --

MR JUSTICE GOSS: Well, I just quoted what you put in your written submission.

MR MYERS: Exactly. It comes from that.

MR JUSTICE GOSS: That's the passage from which it comes.

MR MYERS: It is. But following on from that, we submit it's significant that there are sources of evidence that can far more directly diagnose air embolus, none of which feature in this case. I'm just going to identify this for your Lordship because it isn't as perhaps as in the dark as might otherwise appear from what the experts have said in this case, all of whom put significant store on this article.

Reading on, in fact, from the section that your Lordship has just referred to, a couple of lines down we have:

"The most distinctive sign of pulmonary vascular air embolism..."

I just confirm -- does your Lordship...?

MR JUSTICE GOSS: I'm on page 509.

MR MYERS: About three or four lines below the passage your Lordship referred to. It's the first paragraph, the lengthy paragraph under the table. If your Lordship just tracks that down about five or six lines, you'll come to the description of colour and then three or four lines below that, halfway across the line it begins:

"The most distinctive sign of pulmonary vascular air embolism observed in half of the reported cases is a finding of free air when blood is withdrawn from the umbilical arterial catheter. Columns of air or a frothy mixture of blood and air were often obtained."

In fact what this article does is identify phenomena which do, it seems, directly indicate air embolus, none of which are found in this case. It may be that the circumstances mean they were not found because they couldn't be because these tests weren't conducted, but it wouldn't be right to say there is no way of accurately identifying air embolus.

They go on to say:

"Another related phenomenon observed in two reported patients and in one of our infants was an inappropriately high arterial oxygen concentration recorded from intra-aortic oxygen electrodes in direct contact with gas bubbles."

Then, and this is something that was raised with some witnesses:

"A radiograph is diagnostic and free air may be seen in both the arterial and venous systems as well as in the heart."

And then it refers to the figure at the top of the page, which in fact Professor Arthurs was taken to by me at one point in his evidence in slightly clearer image.

It goes on to say on this topic:

"In 75% of reported cases the radiographs were taken ante-mortem."

Which is an issue that's occupied this court in fact:

"Post-mortem radiographs..."

Pausing there, my Lord, in 75% of reported cases of air embolus, the radiographs were taken ante-mortem. So it would appear it's capable of being diagnostic, depending on the circumstances. Then:

"Post-mortem radiographs need to be interpreted with caution as intravascular air may appear as early as 25 minutes after death."

Of course that has some bearing on what Professor Arthurs has said, but that's not the reason I go to that now. I go to that because there is more direct evidence available in particular circumstances of air embolus.

Now, it may be, for one reason or another, these sources of evidence are simply not available in the cases we're dealing with here. But if that's right, then of course, we respectfully observe, it doesn't follow that the next best alternative is what we have to work with because the next best alternative may

not have sufficient reliability to support the allegation that's made and that is the test. No matter what justifications are given for a paucity of research or experience, that's the test.

My Lord, I'm going to move to the section that deals with Sally Kinsey and then Owen Arthurs. That is not because we don't set significant store by our criticisms of Dr Evans and Dr Bohin.

MR JUSTICE GOSS: No, but there's nothing you wish to add to what's in writing?

MR MYERS: No, nothing I wish to add. That just simply elaborates on what your Lordship has.

MR JUSTICE GOSS: Thank you.

MR MYERS: Professor Kinsey, who was a retired paediatric haematologist, was called to give evidence in particular on the question of discolouration. And we've either cited or included within the argument key factors relating to her evidence that, we submit, mean -- in fact, as she accepted -- that she is no expert on this topic. In fact, that line of questioning concluded -- and we've included in our paragraph 53 -- which was she ended up saying:

"I was asked if I could make a bit more of an explanation about the features of an air embolus. I did say at the time I was not an expert in this area but I was asked if I could do it anyway."

She dealt with how discolouration could happen because of air embolus, referring to Lee and Tanswell and also a paper by the writers Garcia and Mitchell. In fact, that's a paper she cited, a key paper she cited in her statement of 1 November 2022.

But as she accepted, my Lord, that statement dealt with deep sea divers, adults, overweight, as it happens, and comes from a journal called Diving and Hyperbaric Medicine, which establishes:

"Medical treatment in which an ambient pressure greater than sea level atmospheric pressure is a necessary component."

And it is classically what divers have when they come from depths, the decompression sickness that they have.

MR JUSTICE GOSS: The bends.

MR MYERS: Yes, the bends. She accepted -- we have put the references here -- that that isn't of general application, it's a very limited area, and rather like what Dr Bohin had to say about dogs and rabbits and adults, but the more so, certainly so

far as adults are concerned, there is no proper basis for taking that and applying it to neonates in the circumstances of this case, and she didn't suggest there was.

She, rather like the comment that we identify from Dr Bohin -- her evidence on this, in general, is, we respectfully submit, an object lesson of the potential pitfalls here because she's somebody who's a professor of haematology and on the basis that she works with blood and circulation, she was prevailed upon to assist, in effect, to help out with how discolouration could occur with air embolus and in due course be used as diagnostic in this case. And as she accepted, what she had to say about the circulation and gas exchange was of general application, she is no specialist in this, and the material she relies on to talk about discolouration is outside her field of expert and is highly limited, the Lee and Tanswell and then the Garcia report, which is really about something entirely different.

Owen Arthurs, my Lord, Professor Arthurs, is a paediatric radiologist, working at Great Ormond Street. There was, when he first gave evidence, cross-examination of the process he went through to reach the conclusion that gas in a great vessel or vessels post-mortem is unusual. Cross-examination was designed to look at that process critically.

It is helpful, we submit, to keep in mind the factors, in particular at paragraph 19A.5 of that Criminal Practice Direction, when considering what Dr Arthurs said. There's no doubting the diligence with which he went about his study. The paragraph I should say, 19A.5 of the practice direction, is found at pages 1 and 2 of the defence bundle.

MR JUSTICE GOSS: And set out in paragraph 14 of your --

MR MYERS: Yes, my Lord.

MR JUSTICE GOSS: And you've gone through it.

MR MYERS: We have. In many ways, again, illustrations of the problems presented by this evidence emerge with Professor Arthurs. For example, a process which he has conducted, there is no peer review of this, it's one study he conducted with the available material of children, of a relatively limited population, in fact, babies as close as he could find them to those in this case, neonates at Great Ormond Street. We don't doubt his intention to produce research that was as strong as he could make it. But the group that he could draw upon is necessarily, we would submit, limited.

In terms of evidence-based medicine, he agreed it wasn't of the highest order and we submit it's certainly not of the highest

and it would certainly struggle to pass muster in an academic journal, not least because, we submit, there aren't the necessary or controls that would be in place in terms of criteria to be applied at the outset, how things are going to be measured against that peer review, which is important. The size of the sample group, which is very small, to be used as general application in this case.

And of course, against that, in fairness to Professor Arthurs, is the fact that he identified other mechanisms or other causes that could account for the air that was seen in the great vessel or vessels in this case.

So even with a process that, we submit, would fall short of the criteria or the type of criteria in paragraph 19A.5, even at the end of that and interpreting it as generously as possible, it certainly doesn't go to identify air embolus with specificity. We still end up with a 25% chance that it is post-mortem gas of some sort, though he said that's usually where there has been trauma of some sort.

But one bears in mind what's also in this case, the Lee and Tanswell article, which talks about post-mortem has more generally appearing within some minutes, in fact, of death. He also identified other explanations which may be applicable in this case, so we submit that it was a purpose-built study by him for a trial in which he was a witness, about which he came to give evidence about his own study at his own hospital with subjects that he chose that hasn't been peer-reviewed.

That isn't to criticise his intentions, but it's to criticise the method.

Finally my Lord, so far as experts are concerned, Andreas Marnerides, who is a paediatric pathologist and histopathologist, did not in fact, we submit, contribute to the issue that is under consideration here, which is whether or not there is a sufficiently reliable body of expert evidence to apply when air embolus is under consideration. And in fact, when cross-examined about the mechanism of air embolus in a neonate and how it operates, and this is in our paragraph 69, he was either unable or, we have to say, unwilling to explain it because he kept on saying, "I'm not a clinician, I deal with the point of death, I can't explain how this happens", and he wouldn't.

We understand the limits around a professional expert witness and what they can give evidence upon, but we submit that were it to be said that he is an expert on the general sphere of air embolus, it's a very poor starting point that he felt unable to describe how it works in a neonate and he didn't really set

himself out as that. His role was principally to see whether there were alternative causes from the pathology that could account, in individual cases, for death and also whether there were any findings that in an individual case might be capable of supporting air embolus. And the only one of those, in fact, was in the case of [Baby A] when he described microscopic bubbles in slides, some of which had come from the lungs and some of which had come from the brain.

All credit to him, he didn't seek to say that's diagnostic or conclusive, but in light of what the clinicians or the radiology would show, it's something that could be supportive of it. But we have no description of how an air embolus comes to do that or how it would fit in with it. He has identified air, possibly and how that may play a part in one of the charges on this indictment, but he cannot deal with the question of the reliability of the evidence in terms of establishing a theory that can be applied. And given his reliance upon the clinicians, which he was very clear about, however we approach Dr Marnerides, and he only really bites on that one count in this respect, however we approach it he cannot get beyond the quality of the expert evidence on air embolus insofar as it comes from Drs Bohin, Evans and Professor Arthurs. He relies very heavily on what they say and proposes no alternative.

My Lord, those are the principal contentions that we raise with the submission that we make.

In conclusion, we submit that this evidence is fundamental to counts 1, 2, 4 and 16, and on the evidence it is insufficiently reliable when measured against what is required of scientific evidence and when measured against its prominence and significance in this case up to now. It is insufficiently reliable to support conviction on those four counts and, for the same reasons we submit, insofar as it has been used to support the allegation on counts 3, 5, 12, 17 and 20, it should form part of that medical evidence no longer, howsoever that is achieved, which is a different question, but that's where we get to.

If we can assist now, my Lord, we will, otherwise we'll assist after your Lordship's heard the prosecution.

MR JUSTICE GOSS: You will have the right of reply after Mr Johnson has addressed me.

MR MYERS: Thank you, my Lord.

MR JUSTICE GOSS: Mr Johnson, thank you for your written response. I appreciate the constraints of time obviously have an impact.

MR JOHNSON: Yes.

MR JUSTICE GOSS: You should feel free to add or emphasise anything that you wish.

Submissions by MR JOHNSON

MR JOHNSON: Thank you. Can I start with one of the fundamental principles of an English criminal trial.

It's the burden of proof and it's a point that my learned friend raised at the beginning of his submissions by referring to paragraph 13 of the decision of the Court of Appeal in the case of Angela Cannings.

He read the paragraph out and I'll read it again if I may:

"Reverting to the two possible approaches posed in a case like this in a criminal prosecution, we have no doubt that what we have described as a second approach is correct. Whether there are one, two or even three deaths, the exclusion of currently known natural causes of infant death does not establish that the death or deaths resulted from the deliberate infliction of harm. That represents not only the legal principle, which must be applied in any event, but in addition, as we shall see, at the very least it appears to us to coincide with the views of a body of expert medical opinion."

What my learned friend didn't do is to move on to the next paragraph, which I will read at least in part:

"As we have already emphasised, the question in this case was whether there were any crimes at all and whether there were, in the legal sense, any victims of crime. As we shall see, there was no direct evidence and very little indirect evidence to suggest that they were. There was further evidence which tended to suggest that they were not."

So that is an enormous difference to the context of our case.

In our case we have 17 children, not one, two or even three. We have 22 events with which Lucy Letby was intimately associated, all of which, according to the prosecution medical evidence, are unexplained by any possible natural event. But more importantly, we have two cases in which children were poisoned with insulin, and poisoned via bags of TPN or dextrose, which Lucy Letby certainly handled.

So far as [Baby F] was concerned, it was a bag put up at 00.25 on 5 August that was poisoned and she was the person that put it up.

So far as [Baby L] was concerned, she had hung the original bag, which had nothing in it other than its legitimate contents, and at some time between just after midnight on the day she came in on that Saturday morning and the blood sample being taken towards the end of the morning, somebody deliberately put insulin into the bag.

The second bag that was hung for [Baby M] was poisoned as well and that was done at a time when Lucy Letby was there.

So just pausing there, my Lord, and moving back to paragraph 14 of Cannings, what we have here is a case in which there is very real evidence that the woman in the dock is the poisoner, and one has to look at all the other events in that context. That is something which has been singularly ignored by the defence in all the submissions they have made.

But it doesn't end there because [Baby O] was assaulted and he was assaulted at a time when Lucy Letby was his designated nurse. He collapsed at a time when there is no evidence at all to suggest anybody else but she was there. And he sustained liver injuries which Dr Marnerides has told the court, in no uncertain terms, can in effect only be the product of an assault.

But even there, my Lord, it doesn't end, because his brother, his triplet brother, [Baby P], his ET tube dislodged itself at a time when he was paralysed with pancuronium and when [Dr B] and [Dr A] were in the tea room at 12.28 on 24 June 2016. Yet again, Lucy Letby was the designated nurse, Lucy Letby was standing over him when it happened.

And of course, in that context, the jury will bear in mind that [Baby K]'s ET tube found its way, displaced at a time when Lucy Letby was not his designated nurse but the designated nurse had left about 2 minutes earlier, and when it was discovered who was standing over him but Lucy Letby, what was she doing?

Absolutely nothing, in a situation in which the uncontroverted expert nursing evidence was agreed and the uncontroverted evidence suggests that what she was not doing was utterly unacceptable.

So from assaults and poisonings on five children, we have the context for everything that follows and that is the context on which this jury will interpret evidence given by a large number

of doctors that all these other children collapsed, some of them died, for unfathomable reasons.

So my Lord, this case is completely removed from the authorities which my learned friend has quoted. There is very powerful evidence that Lucy Letby has repeatedly attacked neonates in the Countess of Chester Hospital, and she's confessed to it because she has written notes saying, "I killed them. I am evil".

In interview she was asked why she did that and she said it's because her competencies had been challenged, but yesterday we heard from Eirian Lloyd Powell, who was present at the meeting when she in effect was moved, and she used a very emotive word to describe the way Lucy Letby reacted to being moved: it was distraught.

The type of word that most people listening to the evidence in this case would understand being used as an adjective for the parents of these children, but seems out of place, we respectfully submit, to describe the reaction of somebody who was "only" told that she would have to redo her competencies. The reason she was distraught, of course, is because, as Eirian Lloyd Powell told us, she knew she was being accused of having killed these children. It was not her competencies that were being brought into question.

If the jury accepts that evidence from Eirian Lloyd Powell, they will reject Lucy Letby's explanation for her reasoning behind writing, "I killed them". That, my Lord, is quintessentially a matter for the jury. And to suggest, as the defence do, that this is a prosecution that rests solely on the opinions of several doctors that some of these children were killed by an air embolus is a gross distortion of the evidence.

So my Lord, that is the context of this submission and that is why we respectfully submit it is ill-judged, to use a phrase that was used of our counter-submissions.

My Lord, that's the starting point for our response.

I understand that some of the suggestions that we have made in writing are resented. The reason that we have set out in writing at paragraph 2 of our response our understanding of what the submission is about is to put into context the body of our response. And our understanding is that the defence seek to persuade the court that there's no case to answer on counts 1, 2, 4 and 16 because of deficiencies in the air embolus material as it stands at the conclusion of the prosecution case.

I must say, and I'm sure that it's a reflection of my inadequacies rather than the way my learned friend wrote his

submissions and argued them this morning, I still don't really understand the basis of the submission to this extent. One of the things my learned friend said earlier, and it's at paragraph 24 (sic) of the running [draft] transcript that we've got going, is:

"I'm not arguing admissibility."

Well, if the defence are not arguing admissibility, what are they arguing here?

It may be that I have -- I'm quite prepared to accept that I have misunderstood what has been said.

MR JUSTICE GOSS: Sorry, just give me a moment.

(Pause)

Please continue.

MR JOHNSON: In the context of admissibility we understood -- and again I invite correction at a point where I can deal with it if I need to deal with it -- but we had understood that one of the fundamental principles of the submission was that there was -- in effect, air embolus as an area of bogus science. It's what I've called "the sufficiently reliable scientific basis point".

MR JUSTICE GOSS: I don't think Mr Myers is actually suggesting it's a bogus science. That's why I asked him specifically the question: do you accept that there is a respectable -- I didn't put it this way, but there's a respectable body of medical opinion that air emboluses can cause collapses and death essentially? And Mr Myers accepts that. So air embolus itself as a mechanism for collapse and death is not controversial.

MR JOHNSON: Yes.

MR JUSTICE GOSS: That's right, isn't it, Mr Myers?

MR MYERS: Yes.

MR JOHNSON: Good.

MR JUSTICE GOSS: So we're beyond that stage. What, as I understand it, Mr Myers is saying is that the evidence before the jury in this case from the prosecution expert witnesses is insufficient for a jury safely to conclude that in these identified counts air embolus was capable of being a cause of death.

MR JOHNSON: Yes, well, that gives rise to two issues.

First, is that right per se? And secondly, if it is right, what's the consequence? For reasons that I've hinted at but haven't and can't today expand on in great detail, simply because of the pressure of time, we respectfully submit that there is a raft of evidence in the four counts that have been referred to -- it's [Baby A], [Baby B], [Baby D] and [Baby M] -- there's a raft of evidence to support the cases against them.

So just in effect dealing with one of those cases, ex improviso, with [Baby M] we go back to [Baby L], of course, [Baby L] being poisoned. We would say there is clear evidence from which the jury could safely infer that the poisoner was Lucy Letby, and [Baby M] collapsed when, surprise, surprise, Lucy Letby had just had access to his intravenous lines.

In the context of an air embolus case, that fact in itself, when a child is exhibiting signs which the medics who are there are saying, "We have never seen before, save in some of the cases, in other cases in this particular trial", that in itself is evidence from which a jury could reasonably infer that, first of all, Lucy Letby is responsible and, secondly, the mechanism by which she achieved the collapse of the child.

That, of course, chimes -- and moving away from [Baby M] back to [Baby A] at the beginning of June 2015 -- it chimes with evidence that the jury heard yesterday from the interviews, from the overarching interviews, in which Lucy Letby was asked about air embolus and, we would say, she feigned ignorance. And the evidence that she feigned ignorance is the fact that she had undertaken an educational course a couple of weeks earlier, and we have the record or at least part of the record of the course, in which, in her handwriting, signed and dated by her, she has identified that particular issue as being a risk with access to intravenous lines. So the jury in those circumstances, for example, would be entitled to say, "We are quite sure that she wasn't telling the truth in the interview".

Why, if this wasn't a case of air embolus, would she possibly want to lie about her knowledge about air embolus? And the answer, of course, is to distance herself from what went on.

Just returning to the written submissions if I can, please, my Lord, we respectfully submit that many of the -- except insofar as they establish what in effect are trite principles of law, many of these authorities aren't going to assist the court in determining the particular issues that need to be resolved today.

Holdsworth, ironically, is similar to Cannings because there there was positive evidence that militated against the

suggestion that the defendant had committed a crime against the deceased child. We have isolated that in our paragraph 4(a)(iii). In that case the particular child in question suffered from pre-existing brain abnormalities, which predisposed him to the very sort of issue that arose and resulted in his death.

MR JUSTICE GOSS: You needn't dwell on this. Each case is fact-specific.

MR JOHNSON: Quite.

MR JUSTICE GOSS: And the important principle is that expert evidence has to be scrutinised --

MR JOHNSON: Yes.

MR JUSTICE GOSS: -- and at two stages. First of all, is it admissible? And we are past that stage now because it is in evidence.

MR JOHNSON: Yes.

MR JUSTICE GOSS: Secondly, once it is in evidence, is it of such a nature, in the context of all the evidence in the case, such that a jury could not properly safely come to the conclusion that the evidence of the expert -- that they can rely on it?

MR JOHNSON: Exactly.

MR JUSTICE GOSS: That's really what it boils down to.

MR JOHNSON: It does, my Lord. That is precisely the point we were driving at when we deal in paragraph 4(c) with the citation of the well-known case of Galbraith because the legal principles, we would submit, are being confused here in this submission.

We all know, but it's worth repeating, that the judge in a criminal trial can stop a case at the close of the prosecution case when the evidence, viewed as a whole, is weak, vague or inconsistent. That is not authority for the principle which my learned friend seems to be encouraging the court to adopt, which is the judge should exclude a piece, one strand of circumstantial evidence on the basis that it's weak, vague or inconsistent. We don't for a second concede that this evidence, either on an individual case basis or taken as a class of evidence, falls within that description.

Of course, just because in my old age, after all these years, I've never heard of somebody seeking to exclude evidence on that

basis at half time, just because I've never heard of it doesn't mean it can't be done, but I'm not sure that there's any reported authorities to suggest that this is a correct, or even permissible, way of approaching the admissibility of evidence in a trial. The evidence is in. It's for the advocates to argue for and against it and it is for the jury to put it into the balance, to assess it individually, but, more importantly, in the context of all the other evidence, and decide where the global picture takes them.

That's a count-specific approach, if you like. The case-specific approach is the cross-admissibility point, which we've raised in writing. I am not going to deal with it at any great length, but in this context, given that the nature of the defence is innocent coincidence, it is classically a case in which the evidence on one count can and should be taken into account by the jury.

That, of course, is another reason why we would submit that to approach the submission as it has been approached, namely to say the prosecution have put all their eggs in one basket, they have taken up the challenge of proving air embolus and therefore if, looking at air embolus in a vacuum, on a case-by-case basis, the court concludes that that one class of evidence is in some way deficient by measuring it against the stick of the second limb of Galbraith, to suggest that that therefore enables the court to stop a count at half time, we would respectfully submit is -- well, it's an affront to the constitutional role of the jury, in effect.

The evidence is in, it is admissible, there is no unfairness to the accused. Therefore it must be weighed in the balance and your Lordship, we would respectfully submit, can't usurp the function of the jury in a two-stage process: first, by ruling out evidence that's actually in and, secondly, by then deciding that because that one strand of the circumstantial case in an individual count is in some way deficient that the whole circumstances of the case can then be ignored and the count as a whole should fail at that stage. That's why we would respectfully submit that the process that my learned friend encourages the court to take is misconceived.

We have suggested that this application is an abuse of the process of the court and we continue to do so, for two reasons. The first is that there is material on which it is clear that this was within the contemplation of the defence from the very beginning, that if it was a legitimate approach it is a submission that should have been made before the evidence was called, and also because of the submission that your Lordship heard back in January this year concerning the evidence of Dr Evans. There, the admissibility of his evidence was being

challenged and if this was properly arguable it should have been argued at that time.

Otherwise, the points that we made in our written argument relating to Dr Hall are now irrelevant because they were based on what we had understood to be one of the focuses of the submission that, in effect, air embolus was bogus science. That was the purpose of --

MR JUSTICE GOSS: Yes, well, I'm not going to have regard to Dr Hall because it's not evidence in the case. This is a submission that there is insufficient evidence for this case to proceed, so we can forget about Dr Hall.

MR JOHNSON: Yes, of course. I'm just --

MR JUSTICE GOSS: No, no, I understand. You're justifying why you referred to it.

MR JOHNSON: Yes.

MR JUSTICE GOSS: You needn't concern me in relation to that.

MR JOHNSON: Moving on to paragraph 15 of our response, which has been, in effect, repeated orally this morning, or this afternoon, that there is no consistency as to the discolouration claimed to be indicative of air embolus, and in that context, of course, Dr Bohin's evidence was referred to, we submit that's a non-point, not least because there are all sorts of articles saying all sorts of things about discolouration in air embolus.

It's not only Lee and Tanswell that's been referred to in the context of this case, but just putting that to one side or putting the other articles to one side for a moment, Lee and Tanswell has two important -- a number of important points, but two points are that even in that article there are two versions of the discolouration that are seen in air embolus.

The second is the point that I think your Lordship, or it may have been my learned friend, referred to earlier, that in many cases there is no visible cutaneous sign at all. So to rely on the fact that there have been conflicting or different, rather than conflicting different, descriptions of what witnesses have seen at various stages, to rely on that as evidence that undermines a suggestion that these are cases of air embolus doesn't actually measure up to the peer-reviewed learning.

The real point, we would submit, coming out of all these discolouration episodes is that so far as the medical and nursing staff are concerned, they all say, "I've never seen anything like it before and I've never seen anything like it

since", except in some cases, just taking as an example [Nurse A] with [Babies A & B], where she says of [Baby B] on 9 June, "I had actually seen that about 24 hours earlier with [Baby A]".

We submit that the essence of the air embolus type discolouration is the one that has been stressed by many of the medical practitioners: it's transient and often it's migratory and there is no -- I think it may have been Dr Gibbs or Dr Jayaram, I can't remember off the top of my head, I'm afraid, but one of them said that there is no naturally occurring illness or disease that produces visible signs like that.

And yet again, we submit, that is evidence from which a jury could infer, given what they understand about the mechanics of air embolus, the physiological characteristics of it in the sense of the air bubbles moving round the circulation and blocking off parts of the circulation or getting into the heart and stopping the heart, they could, given the evidence they have heard, conclude that these are cases of air embolus because that explains it, nothing else does.

The person that was there was Lucy Letby, often she had just had access to the lines, and it's documented.

Sometimes she was in a room where she had in effect no business to be and, as always, the child collapsed.

Going back, just before I forget, to a further piece of direct evidence which implicates Lucy Letby, we have what came out of her interviews about [Baby C], where having initially said that she'd had nothing to do with him, hadn't been in the room, when the police put her interviews to her -- sorry, when the police in interview put to her her text messages, first of all she accepted that she had been in the room at the time he had collapsed and at the time she had been both frustrated and angry.

So putting that also into the balance of the circumstantial case, we would respectfully submit that there is more than enough here for this jury to act on the evidence that they have heard about air embolus, to weigh it in the balance with all the other circumstances of the case and reasonably to convict Lucy Letby.

My Lord, I don't know whether there are any specific issues that have been raised either by the defence that I have failed to address or issues that Mr Myers has raised this morning that I haven't answered or indeed issues that have occurred to your Lordship that you'd like me to assist on. But other than to say what I have said, in addition to what we have submitted in writing, I'm not sure that there's a great deal further that I

could contribute that will actively promote the cause that I seek.

MR JUSTICE GOSS: Thank you. Yes, Mr Myers.

Reply by MR MYERS

MR MYERS: Three matters to deal with in response if I may.

MR JUSTICE GOSS: Yes.

MR MYERS: The first, which is not novel, but just to emphasise a point, is that the basis for this submission is absolutely plain and set out in our application.

It's plain because it's written there. Paragraph 7 makes it plain this is an application that there is no case to answer on counts 1, 2, 4 and 16 on the basis that your Lordship articulated by way of explanation.

And that's right, it has nothing to do with admissibility and nowhere here does it say that it does.

Paragraph 8 dealt with the position on those counts where air embolus had featured in the evidence, as to which our submission was that if we are correct in our principal submission, there would have to be a direction to the jury, we say, to disregard that evidence or deal with it however the court saw fit.

But there's nothing novel, as it happens, in a court being required to deal with evidence that, with hindsight, is reviewed as evidence that the jury should not have. That wouldn't be a novel situation.

So we're not creating a new jurisdiction, fresh law, in asking the court to hear an application that there is no case to answer. That's plainly what it is and we could not make this application if we hadn't had all the evidence that we've had in this trial.

I pause just to remind myself of the transcripts before your Lordship, which are only a fraction of this evidence, many of which occurred both well after the start of the case, obviously, and after our application concerning Dr Evans.

We are in this position now because of the totality of the evidence and it really, we submit, isn't right to criticise the defence for making an application they are entitled to.

MR JUSTICE GOSS: Well, you are entitled to make the application.

MR MYERS: Yes.

MR JUSTICE GOSS: Whether it succeeds or not is for me to decide but you are not in principle prevented from making the application both of no case in relation to those counts you submit there is no case to answer and to invite me to direct the jury to disregard evidence if I came to a particular conclusion. That's within my power. It may be unusual in the circumstances but I don't think it's beyond my powers.

MR MYERS: Not at all and we're grateful for that. It really isn't, thank you, my Lord.

MR JUSTICE GOSS: So we needn't worry about that.

MR MYERS: Just two other matters then.

MR JUSTICE GOSS: Certainly.

MR MYERS: The first of those is the question of the totality of the allegations and how one impacts upon another. We recognise this may be a matter for further consideration at a certain stage, but it was an opening matter by my learned friend in which he identified five, as I recall, particulars cases, and said that in effect they set the context for this case and this should be reviewed in the light of that.

So far as my note goes, none of those in fact deal with this discrete issue on counts 1, 2, 4 and 16. As I recall, none of the cases he cited were those cases.

It is something the court, we respectfully observe, may have to grapple with in due course, the relationship between the different counts on this indictment, but what we say at this point is that there are multiple factors and issues in play across these counts. This is not a case where there is such a symmetry, such a factual symmetry across the counts that what goes for one will necessarily go for all.

Taking the simplest of illustrations, a string of robberies or a string of sexual offences often exhibit precisely the same characteristics. That arguably is an easy one and it's possible there to move from what is proved on one to being of general application.

In this case there are very different issues across the counts. In some issues there are questions of the quality of medical care generally, on some of them there are questions of the extent to which natural or clinical problems may have impacted upon what took place.

There's not one that applies to everything. And then of course the mechanisms themselves that are alleged are different.

So we submit that we're back, in answer to that, with a fundamental submission we make, which is that on the counts of air embolus the medical evidence is and remains central to them and if the theory or the basis in terms of scientific knowledge for alleging air embolus is insufficiently reliable so that, for instance, that evidence should not be there at all, if one then considers where would those counts be without that evidence, we submit there would be no case on them, no matter what one saw elsewhere on this indictment.

So at this stage our submission is that it is no answer to the submission we make to seek to go to other counts and talk about the strength of them.

The final matter, the reasoning behind which is similar, is the question of those general factual issues, examples of which the prosecution have given, like what is in the notes and what may lie behind the notes and how they may be of general probative application and are matters to consider alongside the medical evidence.

It was put this way, that matters like that, matters like what Eirian Powell had to say or explanations for the notes, or maybe what's said in interview, are quintessentially for the jury. Of course matters like that are and the jury will have to also weigh up the medical evidence. But the theoretical and scientific basis for that medical evidence in itself is not a matter for the jury, that's why the experts come, to explain their opinion, and the basis for that opinion and the jury can evaluate it.

But the medical evidence, if the court forms the view that it is fundamentally unreliable or insufficiently reliable, if on that basis, again, one were to remove that from the equation where it applies, we submit that whatever questions may quintessentially be for a jury with the remaining shreds of evidence in the case or pieces of evidence, they would not be able to provide a basis of guilt where, for example, on these particular counts, air embolus is alleged. They really wouldn't be.

There is no way of getting from a note, with what the prosecution would say is an admission on it, that could apply to any, if it is, any one of the matters on this indictment but no one in particular. There is no way of getting from that to saying: well, that that now becomes generally applicable to everything in answer to the submission that we make. Whatever its relevance, it may have nothing whatsoever to do with the counts under consideration by the court right now, and that's

why we say that it is the medical evidence that remains central to these and, where that is not strong enough to meet the requirements of scientific evidence, and we submit it isn't, then those counts that rely on this area of the evidence are not strong enough to go to the jury, for the reasons we have submitted.

So those are the matters we raise unless there's anything else I can assist with, my Lord.

MR JUSTICE GOSS: No, thank you very much.

I shall consider the respective submissions. What I'm going to do is to break off now and sit again at 2.05, by which time I will have reached my decision or decisions. But I will then announce what my decision is and then provide reasons for my decision in writing at a later time rather than trying to give an ex tempore judgment, but address the matters in some detail. Not exhaustive detail, but in some detail, in writing --

MR MYERS: We understand, my Lord, thank you.

MR JUSTICE GOSS: -- in due course. So what I would like to do so then is, after I have done that, we will then move on to the issue in relation to the -- your representations so far as arrangements for the defendant giving evidence are concerned. We'll address those at 2.05 or 2.06. Thank you very much.

MEMBER OF THE PRESS: Can the press be provided with copies of your ruling?

MR JUSTICE GOSS: Certainly, you will do. But you're not going to get it today. You'll know what the decision is and I will say, "For reasons that will be put into writing", and you will be provided with copies. Certainly you should have copies. But of course it's not material that can be published at all until the conclusion of the trial. By then, a lot else will have happened. Thank you very much. You may rest assured in that regard.

Thank you very much. 2.05.

(12.53 pm)

(The short adjournment)

(2.05 pm)

MR JUSTICE GOSS: Yes, Mr Myers -- do you want to say something?

MR MYERS: No, I apologise my Lord, I was ready to deal with something else, but of course I shall wait for a moment.

MR JUSTICE GOSS: Yes. For reasons that I shall put into writing and distribute next week, I refuse the applications, the two linked applications that are made in relation to the submission of no case to answer and giving directions to the jury to ignore the expert evidence in relation to air embolus on the other counts.

As I say, it'll be set out in detail in writing.

MR MYERS: We understand that, my Lord.

Housekeeping

MR JUSTICE GOSS: So thank you again, I know I've already acknowledged your document that you provided on 4 April, actually, dated 4 April, Mr Myers. As I indicated to you yesterday, I am basically content with the proposals. Perhaps we should just go through them one by one.

MR MYERS: Yes. If I may just, by way of preliminaries, set out a little bit, just very briefly, about the basis but not the detail of that that as set out in the report, just to introduce it, my Lord, if I may.

MR JUSTICE GOSS: Yes.

MR MYERS: We are grateful for the court's reception of this and we're grateful to the court, I should say, for the assistance throughout the case, so far as Ms Letby and her attendance and her conduct in the trial has been concerned, and we're also most grateful to the cell staff, accompanying her to and from court. They have had a significant part to play in assisting with the process.

We know the court understands that from every perspective Ms Letby has to be able to deal with the evidence as well as she is capable and we've set out the material we rely upon, I don't go through that now, it's in the report.

When we come to consider the arrangements, the only matters that I advert to specifically are concerns as to what's described as hypervigilance and hypersensitivity to aspects of her surroundings. That's something that we are concerned with and we know the court will seek to assist us with and Ms Letby with to ensure there is no disruption from that.

Again, being careful not to go beyond where I need to for what we're dealing with here, to monitor closely how she deals with the process that she is going to be dealing with, which on any view would be a demanding process for anybody, and there

certainly is material to indicate a marked degree of -- potential degree of fragility. So we're most anxious that that be monitored closely because of the potential for significant problems in certain circumstances. I don't go into more detail than that because I know that your Lordship has the submissions and the material that goes with them.

MR JUSTICE GOSS: Which I have obviously read and take account of.

MR MYERS: Having set that out, can I go through the measures, just to deal with them -- and in fact, if I may, can I deal with what appears at the end because it's something that, as it stands there, may cause more or may potentially cause more concern than would be necessary. That's (x) and (xi) at the end of paragraph 33, which refers to contact with legal representatives during the giving of evidence. May I put the court's mind at rest as to where that lies because that would be a most extraordinary course to take.

It's not one that, at the outset, we are seeking at all, but your Lordship will see the paragraph that follows that talks about contingencies and having in mind any issues that may develop. And bearing in mind the material that we had received by way of reports and what had been said, it seemed appropriate just to raise the issue for the court in terms of forewarning it and monitoring how the defendant is as to what may be required in extremis or depending upon the way events go.

But it's not something we would seek to persuade the court needs to take place now and if we try to, we anticipate it would probably be unsuccessful, so we don't.

MR JUSTICE GOSS: That was the only one that stuck out in my mind. It would be wholly exceptional. But I understand exactly what you're saying: we needn't go into it because you know what I'm saying, I know what you're saying. Should particular circumstances arise then consideration can be given to how to address those circumstances.

MR MYERS: That's why I went there first.

MR JUSTICE GOSS: Exactly. That's absolutely fine.

MR MYERS: Thank you.

MR JUSTICE GOSS: But then you proceed as you wish to in relation to the remainder.

MR MYERS: The first point that we ask the court to consider, and it's something your Lordship's made reference to already, is the

sessions of evidence and breaks enabling the defendant to cope with the demands of giving evidence. As to that, there is a matter that I have been informed of via the cell staff, for which I'm grateful. It's this: that in fact breaks that are longer than 15 minutes are very -- will result in the defendant then having to be taken downstairs, with various arrangements that have to be made because of her categorisation, which can in fact be more disruptive or less helpful maybe than a break of about 15 minutes.

MR JUSTICE GOSS: Right. I understand that. It doesn't surprise me. It also means that, as I understand it, she would then be essentially in the back of the court and in a safe space --

MR MYERS: Yes.

MR JUSTICE GOSS: -- and not coming into contact with anyone else and not wasting time going up and down and so on and so forth. So fifteen-minute breaks then.

MR MYERS: That would be helpful. Then the defendant is within the area of the court, albeit just outside the court, and it's less disruptive.

MR JUSTICE GOSS: What I'm going to say to, Mr Johnson, is if you wish to contribute to any of the proposals at any time feel free to do so. But fifteen-minute -- a mid-morning fifteen-minute break and a mid-afternoon fifteen-minute break.

MR JOHNSON: Yes.

MR MYERS: We can see how matters proceed, but I'd ask your Lordship to consider whether the evidence could be kept within about hour blocks, as we move along. We can monitor that.

MR JUSTICE GOSS: Yes, exactly. I'm not sticking rigidly to the exact hour, but as we do conventionally, either just slightly under the hour if it's a convenient point to break or slightly over the hour. But basically, two one-hour sessions in each half of the day.

MR MYERS: Yes, thank you, my Lord.

Just pausing there, when dealing with arrangements and the custody element, from the cell staff there will be arrangements necessary for the moving to and fro in court. One matter that it occurs to us is necessary to consider is whether the defendant is in position before, for instance, the public are in court. A bit like the screens being put up and taken down. It may well be a matter that the court has already considered or been asked to consider.

MR JUSTICE GOSS: Yes. I think the answer is going to be yes.

MR MYERS: Well, I don't go into that any further at this point but --

MR JUSTICE GOSS: So that will have to be factored in.

MR MYERS: Rather like the screens, in other words.

MR JUSTICE GOSS: Exactly, yes.

MR MYERS: Very well, thank you, my Lord.

MR JUSTICE GOSS: In other words, the court will be closed until a point when everyone can come into court.

MR MYERS: Thank you.

MR JUSTICE GOSS: I will address that specifically next week.

MR MYERS: The second matter -- in fact originally we'd asked whether the defendant may have, in fact, papers copies of the sequence of events charts. Having considered the matter further, in fact very recently, the request we would make is whether she can simply take the iPad that she has used. Having used the A3 files myself and knowing I will be using them, they can be quite cumbersome with a good deal of space around and it may be less help than more help to deal with those. But if the defendant could have use of the iPad in the course of the evidence, then if it is necessary for her to refer to whatever is being dealt with, she can, if that creates no impediment.

MR JUSTICE GOSS: I'll just check first with Mr Johnson.

MR JOHNSON: Absolutely no objection.

MR JUSTICE GOSS: I'm then going to check with the senior officer. Will that be acceptable? All right.

Thank you.

MR MYERS: We're grateful, thank you.

MR JUSTICE GOSS: She can rest assured she will have access to an iPad.

MR MYERS: The next two matters, items 3 and 4, I approach with appropriate circumspection because they deal with the way that the evidence is approached and obviously a particular view as to how my learned friend or the prosecution deal with it. It's not

something I elaborate upon here because I recognise it's unnecessary for me to do so, but I leave it with the court and my learned friend, and of course we shall see how the defendant is. But I simply ask that it be kept in mind, from whatever angle we approach the evidence, it is a huge task to deal with, the evidence in this case. She'll be the only witness who will give evidence in the length and for the variety of areas that she will do, and with that in mind, we ask that it be approached in a way that does accommodate that difficulty or that challenge, let me put it that way.

We appreciate the difference between how questions have to be for the purpose they are designed to serve and the point at which potentially there may be issues that are entirely collateral to the main purpose of questioning. We leave it to the court and my learned friend for that to be kept in mind, as it would with any witness.

MR JUSTICE GOSS: Well, I will certainly, obviously, keep a very close eye on her whilst she's giving evidence.

I'm sure we can rely on Mr Johnson when it comes to --

MR JOHNSON: I state publicly, there is no value to the prosecution in confusing anybody.

MR JUSTICE GOSS: Absolutely. If I think there is any confusion or it's apparent that the witness is tiring and needs a break, I will just intervene at that point.

MR MYERS: We don't regard it as necessary for us to elaborate upon that, but we just state it for the reassurance of all.

The next two matters which I identify and which is the movement around the courtroom is really a matter we leave with your Lordship. The arrangements to be made for the way the court will sit with the defendant has already been considered. It's always undesirable for there to be unnecessary movement in and out of court and we simply observe there are aspects of Ms Letby's condition which, we submit on the basis of the material we have, make her uniquely sensitive to noises, movements, things that distract or unsettle. That will probably become apparent when she gives her evidence, but we ask that there be as little interruption around her as possible. That would be the case anyway but it is likely to be more disruptive here than it might in other cases.

MR JUSTICE GOSS: It's always very distracting in any event whenever anyone is giving evidence when there's movement in and out of the court. And you will have seen for the months that we're all been together that the usher has been absolutely

fastidious, if someone wants to come into court, about going and addressing them. He will do that as well and continue to do that. So it will be made plain that people -- I won't be having coming and going and if people want to leave, they'll leave for the rest of the session.

MR MYERS: We're grateful to Mr Stansfield, of course.

MR JUSTICE GOSS: It has been, in fact, apart from when someone's had a coughing fit and has had to go, it has been very well ordered and will continue to be very well ordered.

MR MYERS: We are grateful, my Lord. Item 7 I can raise with my learned friend; it perhaps doesn't need me to go into the details there. It just relates to matters which may impact upon the defendant's sense of well-being. That is item 7 on page 8. That arrangement can be made between prosecution and defence as to who sits where.

MR JUSTICE GOSS: I don't think it's going to arise.

MR JOHNSON: It won't because if the court is constituted once the defendant is in place, as long as she doesn't turn round to look at people, she won't see anyone.

MR JUSTICE GOSS: Exactly.

MR MYERS: Thank you.

MR JUSTICE GOSS: There is an issue or I was led to believe there is an issue about the screen. Is that right?

MR MYERS: I'm grateful to Mr Maher, who tells me the screen will be turned off in fact.

MR JUSTICE GOSS: That's what I was going to confirm. That will be done, yes. It'll be dimmed. You won't be able to see an image. All right.

MR MYERS: Thank you.

MR JUSTICE GOSS: Thank you.

MR MYERS: Item 8 makes reference to the comforters that that Ms Letby has. She would be grateful, and on her behalf we would be grateful, so long as they create no issue and we don't see how they can, that she at least has those with her. They have played a part so far at every step of the way.

MR JUSTICE GOSS: Mr Johnson, anything to say to that?

MR JOHNSON: No, thank you.

MR JUSTICE GOSS: Yes, certainly she may have her comforters.

MR MYERS: Item 9. All of these matters of important. This is a matter which is and has proved to be quite crucial with Ms Letby, that she can have contact with the court liaison and diversion lead practitioner, who has been Ms Santolini(?) to date. It has been of enormous assistance to her and, as a result of that, to us and thereafter to the court, that Ms Letby has been as able to cope with matters as she has and had the support that she has. Ms Santolini is a professional who deals with this, has been of enormous benefit to her with that, and we would be grateful, so far as it can be arranged, that contact continues, merely for the purpose of support, as it has been throughout, between Ms Letby and Ms Santolini. And so we would be grateful if your Lordship were to consider that and assent to that taking place. It doesn't involve the evidence and will not involve the evidence, it's purely for support.

MR JUSTICE GOSS: Mr Johnson?

MR JOHNSON: Nothing to say, thank you.

MR JUSTICE GOSS: Well, again, I accede to that request.

I will say this to Ms Letby now and I will say it once she commences her evidence, that of course she must not talk to anyone about any of the evidence in the case, and that includes the escorting staff or Ms Santolini, once she has started her evidence. And that embargo remains in place until she has completed her evidence, which is going to be a long time. She can talk about welfare issues, obviously, but not about the evidence.

MR MYERS: She will understand that and will abide by that.

We're grateful for those measures, my Lord.

I emphasise in conclusion that, as with any defendant, but for various reasons especially for Ms Letby, the support she's had to date has been crucial in coping with the process she is going through, and the protracted and at times fragmented nature of it, as it now comes closer, will put its own strains at a time when there's perhaps less support than there has been.

So certainly we will observe very closely how matters proceed and we know the court will. And we are most grateful, as she will be, for every piece of assistance she can have to deal with what is going to take place next.

We will keep the court informed if there's any matter that causes us concern, my Lord.

MR JUSTICE GOSS: As is apparent from what I have acceded to, I am obviously anxious to ensure that she is able to give her best evidence. It goes without saying that it will be a stressful time for her. I have to observe the rules of the court, and they will be observed, and I know you will have explained them all to her. They will be observed. But in relation to the matters that we've canvassed, I am quite content to assist her and therefore the trial process and therefore the interests of justice --

MR MYERS: Yes.

MR JUSTICE GOSS: -- in that way.

MR MYERS: We're grateful for that, my Lord, thank you.

Unless there's anything else I can assist with, those are the matters we wish to raise.

MR JUSTICE GOSS: Thank you very much, Mr Myers.

Anything, Mr Johnson?

MR JOHNSON: No, thank you, my Lord.

MR JUSTICE GOSS: Thank you very much indeed. Obviously, if anything arises you will, I know, alert me to it.

So on Tuesday, when we assemble, of course the jury will now be coming in, expecting to recommence on Tuesday morning, and then the court will sit at 10.30, barring some unforeseen circumstance arising, which it has done in the past, and then we'll take it from there.

MR MYERS: Yes, my Lord, thank you.

MR JUSTICE GOSS: She will know that it will just be Tuesday. Wednesday and Thursday will be rest days for her, which may not be a bad thing after the first day, and then we'll proceed thereafter.

MR MYERS: We're grateful, my Lord, thank you. May we have the opportunity to speak with Ms Letby before we conclude today?

MR JUSTICE GOSS: Certainly, as far as I'm concerned, yes.

The senior escorting officer, as usual, is very accommodating in that regard.

MR MYERS: Thank you.

MR JUSTICE GOSS: And I express my thanks for all the assistance that the court has been provided with.

Thank you.

Nothing else then?

MR MYERS: My Lord, no.

MR JUSTICE GOSS: The court will therefore stand adjourned until 10.30 on Tuesday morning. Thank you very much.

(2.24 pm)

(The court adjourned until 10.30 am on Tuesday, 2 May 2023)